

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 3 Randomized Open-Label Study of Adjuvant Pembrolizumab With or Without MK-2870 in Participants With Resectable Stage II to IIIB (N2) NSCLC Not Achieving pCR After Receiving Neoadjuvant Pembrolizumab With Platinum-based Doublet Chemotherapy Followed by Surgery
Short Title:	A Study to Assess Efficacy and Safety of Pembrolizumab With or Without Sacituzumab Tirumotecan (MK- 2870) in Adult Participants With Resectable Non Small Cell Lung Cancer (NSCLC) Not Achieving Pathological Complete Response (pCR) (MK-2870-019)
Protocol Number:	265110105053023897
NCT Number:	NCT06312137
Sponsor Name:	Merck
Investigational Product:	cisplatin
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Disease-free survival (DFS) as assessed by Blinded Independent Central Review (BICR): DFS is defined as the time from randomization to any recurrence (local, locoregional, regional or distant), occurrence of new primary NSCLC, or death due to any cause, whichever occurs first.
Secondary Obj.:	1. Overall Survival (OS) 2. Distant metastasis-free survival (DMFS) as assessed by investigator 3. Disease-Free Survival (DFS) as assessed by investigator

2.2 Background & Rationale

This study will assess if adding sacituzumab tirumotecan with pembrolizumab after surgery is effective in treating NSCLC for participants not achieving pathological complete response. The primary hypothesis of this study is sacituzumab tirumotecan plus pembrolizumab is superior to pembrolizumab monotherapy with respect to disease free survival (DFS) as assessed by blinded independent central review (BICR).

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	780

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

* Has histological or cytological confirmation of squamous or nonsquamous non-small cell lung cancer (NSCLC),

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resectable clinical Stage II, IIIA or IIIB (with nodal involvement \[N2\]) per AJCC eighth edition guidelines

- * Has confirmation that either epidermal growth factor receptor (EGFR)-directed or anaplastic lymphoma kinase (ALK)-directed therapy is not indicated as primary therapy
- * Is able to undergo surgery based on opinion of investigator after consultation with surgeon
- * Is able to receive neoadjuvant pembrolizumab and platinum-based doublet chemotherapy
- * Applies to screening for the adjuvant period only, before randomization: Has not achieved pathological complete response (pCR) at surgery by local review of pathology.
- * Applies to screening for the adjuvant period only, before randomization: Tumor tissue sample from surgical resection has been provided for determination of programmed cell death ligand 1 (PD-L1) and trophoblast cell surface antigen 2 (TROP2) status by central vendor before randomization into the adjuvant period
- * Applies to screening for the adjuvant period only, before randomization: Confirmed to be disease-free based on re-baseline radiological assessment as documented by contrast enhanced chest/abdomen/pelvis computed tomography (CT) (or magnetic resonance imaging (MRI)) within 28 days before randomization
- * Participants who have AEs due to previous anticancer therapies must have recovered to ?Grade 1 or baseline. Participants with endocrine-related AEs who are adequately treated with hormone replacement are eligible
- * Human immunodeficiency virus (HIV)-infected participants must have well controlled HIV on antiretroviral therapy (ART)
- * Participants who are hepatitis B surface antigen (HBsAg) positive are eligible if they have received hepatitis B virus (HBV) antiviral therapy for at least 4 weeks, and have undetectable HBV viral load at screening
- * Participants with history of hepatitis C virus (HCV) infection are eligible if HCV viral load is undetectable at least 4 weeks before the start of study intervention

4.2 Exclusion Criteria

- * Has one of the following tumor locations/types:
 - * NSCLC involving the superior sulcus
 - * Large cell neuro-endocrine cancer (LCNEC)
 - * Sarcomatoid tumor
 - * Diagnosis of SCLC or, for mixed tumors, presence of small cell elements
- * Has Grade ?2 peripheral neuropathy
- * Has history of documented severe dry eye syndrome, severe Meibomian gland disease and/or blepharitis, or severe corneal disease that prevents/delays corneal healing
- * Has active inflammatory bowel disease requiring immunosuppressive medication or previous history of inflammatory bowel disease
- * Has uncontrolled, significant cardiovascular disease or cerebrovascular disease, including New York Heart Association Class III or IV congestive heart failure, unstable angina, myocardial infarction, uncontrolled symptomatic arrhythmia, prolongation of QT corrected for heart rate by Fridericia's cube root formula (QTcF) interval to \>480 ms, and/or other serious cardiovascular and cerebrovascular diseases within the 6 months preceding study intervention
- * Has received prior neoadjuvant therapy for their current NSCLC diagnosis
- * Has received prior systemic anticancer therapy including investigational agents within 4 weeks before the first dose of study intervention

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- * Has received prior radiotherapy within 2 weeks of start of study intervention, or radiation-related toxicities, requiring corticosteroids
- * Has received a live or live-attenuated vaccine within 30 days before the first dose of study intervention. Administration of killed vaccines is allowed
- * Has received an investigational agent or has used an investigational device within 4 weeks prior to study intervention administration
- * Has a diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy (in dosing exceeding 10 mg daily of prednisone equivalent) or any other form of immunosuppressive therapy within 7 days prior the first dose of study medication
- * Has a known additional malignancy that is progressing or has required active treatment within the past 5 years
- * Has an active autoimmune disease that has required systemic treatment in the past 2 years
- * Has a history of (noninfectious) pneumonitis/interstitial lung disease that required steroids or has current pneumonitis/interstitial lung disease
- * Has an active infection requiring systemic therapy
- * Is an HIV-infected participant with a history of Kaposi's sarcoma and/or Multicentric Castleman's Disease
- * Has a concurrent active Hepatitis B (defined as HBsAg positive and/or detectable HBV deoxyribonucleic acid (DNA)) and Hepatitis C virus (defined as anti-HCV antibody (Ab) positive and detectable HCV ribonucleic acid (RNA)) infection
- * Has a history of allogeneic tissue/solid organ transplant
- * Has not adequately recovered from major surgery or have ongoing surgical complications
- * Severe hypersensitivity (?Grade 3) to study intervention, any of its excipients, and/or to another biologic therapy

11. Study Identifiers & Governance

Primary ID: 265110105053023897

Registry Number: NCT06312137

Sponsor: Merck

Study Title: A Study to Assess Efficacy and Safety of Pembrolizumab With or Without Sacituzumab Tirumotecan (MK- 2870) in Adult Participants With Resectable Non Small Cell Lung Cancer (NSCLC) Not Achieving Pathological Complete Response (pCR) (MK-2870-019)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____